

DEPARTMENT OF RADIODIAGNOSIS

Radiology Report

Register No.: 22037414

Report No.: R275793

Patient Name: GOURI DOMU HARIKANTH

Age: 68 Sex: FEMALE

Bill/Advice No.: 25B0003517

Observed Date & Time: 04-08-2025 11:44 PM



THORAX

History: C/O cough with expectoration. K/C/O carcinoma oesophagus, S/P radiotherapy.
Protocol: Plain and contrast axial images of the thorax and abdomen acquired with slice thickness of 5mm from thoracic inlet to pelvis. 3D reformations done and were viewed in soft tissues and bone windows.

Findings:

THORAX:

Multiple necrotic lymphnodes noted in left infraclavicular, prevascular, upper and lower paratracheal, paraaortic, subaortic, periesophageal and subcarinal stations, largest measuring 17 x 26 mm in lower paratracheal station.

Fibroatelectatic changes noted in bilateral upper lobe apices, anterior segments of upper lobes and left lower lobe basal segments.

Subpleural nodule measuring 6 x 6 mm noted in right upper lobe apical segment.

Left sided moderate pleural effusion noted with maximum thickness of 2.7 cm with atelectasis of underlying basal segments.

Cardiomegaly noted (CT ratio = 0.6).

Mild circumferential enhancing wall thickening of infracarinal thoracic esophagus noted measuring maximum thickness ~ 5 mm for ~ 4 cm length. Anteriorly the fat planes with posterior surface of left atrium and left inferior pulmonary vein appears to be maintained. Fat planes with descending thoracic aorta, vertebra are maintained.

Both lung fields are normally aerated and are applied to the chest wall on all sides.

Pulmonary structure is normal and shows normal vascular markings.

The hilar region on each side is unremarkable and the main bronchi appear normal.

Major intrathoracic vessels and imaged portions of supraaortic vessels are unremarkable.

Degenerative changes in visualized spine.

Hypodense nodules in both lobes of thyroid. *Advised USG correlation.*

ABDOMEN & PELVIS:

Liver: is normal in size, shape, attenuation and enhancement pattern. Intrahepatic biliary radicals are not dilated. Portal vein and common bile duct are normal. **Non-enhancing hypodense cyst measuring 14 x 13 mm noted in segment VIII.**

GB: is well distended. No evidence of radiodense calculi /peri-cholecystic fluid or mass lesion noted.

Spleen: is normal in size and normal in attenuation and enhancement pattern.

Pancreas: is normal in size, shape, attenuation and enhancement pattern.

Kidneys: Both the kidneys appear normal in size, attenuation and enhancement pattern. Corticomedullary differentiation is maintained. **Simple cortical cyst measuring 77 x 61 mm noted in the interpole of left kidney.**

Right sided mild hydroureteronephrosis and left sided moderate hydroureteronephrosis noted.

Supra-renals: The adrenal glands appear normal. No focal enhancing mass lesion seen.

Bowel: **Mild circumferential wall thickening with maintained mural stratification of cardia, fundus and body of stomach measuring maximum thickness 8.6 mm.**

Small & large bowel appears normal. Mesentery and omentum appear normal.

UB: **Urinary bladder is well distended. Wall is regular. Base of bladder and bilateral vesicoureteric junction appears to be elongated and displaced inferiorly with cystic dilatation distally.**

Psoas muscle on both sides appears normal.

Uterus and Ovaries: Uterus is normal in size, attenuation and enhancement pattern. No evidence of SOL within. The endometrium is normal. **Cervix and lower uterine segment are displaced inferiorly.**

Both the ovaries appear atrophied.

Others: Vascular structures are normal in caliber.

Well defined heterogeneously enhancing lesion with necrotic areas within, measuring 28 x 39 x 41 mm (AP x TR x CC) noted in the left gastric station. Similar necrotic lymph nodes noted in paraaortic, paracaval, bilateral common iliac and external iliac levels, largest measuring 13 x 8.8 mm in paraaortic region.

No evidence of free fluid in peritoneal cavity.

IMPRESSION:

K/C/O carcinoma oesophagus, S/P radiotherapy. Present CECT study reveals-

- Necrotic mediastinal, left infraclavicular, left gastric and retroperitoneal lymphadenopathy.
- Left sided moderate pleural effusion with atelectasis of underlying basal segments.
- Cardiomegaly.
- Mild circumferential enhancing wall thickening of infracarinal thoracic esophagus as described.
- Mild circumferential wall thickening with maintained mural stratification of stomach- Likely inflammatory.
- Uterine prolapse with cystocele, right sided mild hydroureteronephrosis and left sided moderate hydroureteronephrosis.

As compared to previous CT dated 23/02/2024, there is significant reduction in the primary esophageal thickening with increase in number and size of regional lymphnodes and new onset metastatic lymphadenopathy. RECIST 1.1 criteria Response: Progressive Disease (PD).

Dr Sreekar
Resident


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